



CANDACORT CREAM

NAME AND STRENGTH OF ACTIVE SUBSTANCES : Clotrimazole 1% w/w and Hydrocortisone 1% w/w.

PRODUCT DESCRIPTION : A white cream.

Other ingredients are Chlorocresol, White Soft Paraffin, Liquid Paraffin, Propylene Glycol, Macrogol Cetostearyl Ether, Cetostearyl Alcohol, Glycerin and Purified Water.

PHARMACODYNAMICS : Clotrimazole is a broad-spectrum imidazole with potent anti-fungal against dermatophytes, including Epidermophytes, Microsporum, Trichophyton spp. and Malassezia furfur, and Candida species. It permeates the chitin of the fungal cell wall and increases the membrane permeability to various intracellular substances. Hydrocortisone is a topically active corticosteroid which has potent anti-inflammatory, anti-pruritic, and vasoconstrictive actions.

PHARMACOKINETICS : When applied topically, clotrimazole penetrates the epidermis but there is little if any systemic absorption. It is metabolized in the liver to inactive compounds and excreted in the faeces and urine. When hydrocortisone is administered topically, particularly under occlusive dressings or when the skin is broken, sufficient corticosteroid may be absorbed to give systemic effects. Corticosteroids are extensively bound to plasma proteins. Only unbound corticosteroid has pharmacological effects or is metabolized. They are metabolized in the liver and tissues, and the metabolites are excreted in the urine.

INDICATIONS : For the topical treatment of fungal and candidal infections, when these are accompanied by pronounced inflammatory symptoms and itching.

CONTRAINDICATIONS : Hypersensitivity to Clotrimazole, Hydrocortisone or any component in the base. The use of corticosteroid is contraindicated in most viral infections of the skin and also tuberculosis and acne rosacea.

ADVERSE EFFECTS : Infrequent local adverse effects of Clotrimazole and Hydrocortisone include stinging, burning, oedema, desquamation, pruritus, and urticaria. Systemic adverse reactions, such as vision blurred, have also been reported with the use of topical corticosteroids.

PRECAUTIONS/WARNINGS : If irritation or sensitivity occurs, treatment should be discontinued. Do not use on children under 2 years of age except under the advice and supervision of a doctor. Avoid contact with eyes and mucous membranes (for external use only). Advise to consult doctor if no improvement is seen in 4 weeks (for athlete's foot) or 2 weeks (for jock itch).

Visual disturbance may be reported with systemic and topical corticosteroid use. If a patient presents with symptoms such as blurred vision or other visual disturbances, the patient should be considered for referral to an ophthalmologist for evaluation of possible causes which may include cataract, glaucoma, or rare diseases such as central serous chorioretinopathy (CSCR) which have been reported after use of systemic and topical corticosteroids.

INTERACTIONS WITH OTHER MEDICAMENTS : Unknown.

ROUTE OF ADMINISTRATION : Topical.

RECOMMENDED DOSAGE : Wash and dry affected area. Apply sparingly to the affected parts and surrounding areas and rub in twice daily, morning and evening or as directed by your doctor or pharmacist. Regular application is essential for successful treatment. Once the inflammatory symptoms have disappeared treatment may be continued with Candazole Cream, if preferred.

Treatment should be continued for two weeks after all clinical signs have disappeared. Attention to hygiene is important in the management of candidiasis, particularly of the folds and the feet. If symptoms persist, please consult your prescriber. This preparation is for the treatment of your current condition only. Do not allow use by any other person and dispose off unused material when your treatment is complete.

PREGNANCY AND LACTATION : Unknown.

OVERDOSE AND TREATMENT : Corticosteroid applied to the skin can be absorbed in sufficient amounts to produce systemic effects such as hypothalamic-pituitary-adrenal axis suppression, manifestation of Cushing's syndrome, hyperglycaemia and glucosuria. Tests which may be helpful in evaluating hypothalamic-pituitary-adrenal axis suppression include urinary free cortisol test and ACTH stimulation test. If the hypothalamic-pituitary-adrenal axis suppression is found, then the drug should be withdrawn, frequency of application reduced or a weaker steroid used. Supplemental systemic corticosteroids may be required if signs and symptoms of steroid withdrawal occur.

PACKING : 5g and 15g aluminium tubes. 450g in polyethylene jars. (For export only). Not all pack size may be available locally.

STORAGE : Keep container well closed. Protect from strong light. Store below 30°C.

For external use only. Keep out of reach of children.

Recommended shelf-life: 3 years.

MANUFACTURED BY :

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